

RACMA Position Statement: Fix the NHRA

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Overview

This position statement outlines RACMA’s formal stance on the structural reform of the National Health Reform Agreement (NHRA). It is supported by a comprehensive series of qualitative and quantitative studies identifying the drivers of “Strategic Gaming” in the Australian public hospital system.

Key Recommendations

1. **Transparency Surges:** Real-time data publication to narrow information asymmetry.
2. **Cap Reform:** Aligning the 6.5% Commonwealth growth cap with actual input costs.
3. **Governance Shift:** Moving from performative documentation to High Reliability principles.

Target Audience

- RACMA Board and Fellowship.
- Commonwealth and State Health Ministers.
- National Health Funding Pool stakeholders. # RACMA Position Statement: Fixing the National Health Reform Agreement (NHRA) **Author:** Dylan A Mordaunt

Executive Summary / Key Position

The Royal Australasian College of Medical Administrators (RACMA) believes the current National Health Reform Agreement (NHRA) must be fixed. The way hospitals are funded today encourages “Strategic Gaming” rather than genuine patient care. Current rules focus on old data and complex caps. This forces hospital managers to focus on meeting metrics rather than improving safety. RACMA calls for a shift toward real-time transparency. We must move away from slow audits and toward high-quality, immediate data. This change will help hospitals focus on being high-reliability organizations that put patients first.

Background / Context

Since 2011, Australian public hospitals have used Activity Based Funding (ABF) {Council on Federal Financial Relations, 2011 @NHRA_2011 #106}. This system pays hospitals based on the work they do. However, the system relies on old information and complex funding limits. This has created problems.

New research shows that the current rules lead to “Strategic Gaming” (Mordaunt, 2025). This happens when hospitals feel forced to adopt “performance masks.” They focus on making their data look good to stay financially stable {DiMaggio & Powell, 1983 @DiMaggio_1983 #110}. Common signals like safety indicators (PSI-90) are often too “noisy” or slow. This makes it hard to see the real quality of care.

Key Issues

1. **Too Much Distance:** There is too much distance between the Federal government (who pays) and the local hospitals (who provide care). This creates a gap in understanding.
2. **Slow Information:** It takes 12 to 24 months to finish funding checks. This delay makes managers focus on short-term money rather than long-term safety.
3. **Confusing Rules:** Many rules in the NHRA are intentionally vague. This allows for cost-shifting and data manipulation.
4. **Bad Regulation:** Current audit models create heavy paperwork. They do not necessarily make hospitals safer or more efficient {Duckett, 2021 @Duckett_2021 #105}.

Recommendations / Call to Action

RACMA recommends these changes to the NHRA: * **Move to Real-Time Data:** Stop relying on old audits. Publish clear, immediate data through **automated extraction** to ensure transparency without increasing documentation burden. * **Fix Funding Limits:** Reform the **6.5% Commonwealth growth cap** to ensure hospital funding keeps up with the actual cost of care. This removes the pressure to “game” the system. * **Make Rules Clear:** Remove vague language from the Agreement. This will stop hospitals from trying to shift costs between different funding pools. * **Focus on Care, Not Paperwork:** Change accountability rules to support patient safety instead of just documentation.

References

1. Council on Federal Financial Relations. (2011). *National Health Reform Agreement*.
2. DiMaggio, P. J., & Powell, W. W. (1983). The iron cage revisited: institutional isomorphism and collective rationality in organizational fields. *American Sociological Review*, 48(2), 147-160.
3. Duckett, S. (2021). Vicious cycles: hospital bed block and the National Health Reform Agreement. *Medical Journal of Australia*, 214(8), 345-346.
4. Mordaunt, D. A. (2025). Structural Analysis of the National Health Reform Agreement: A Game Theoretic Mapping of Strategic Gaming Incentives. *Preprint*.